



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 548/11

BEFORE: S. Ryan: Vice-Chair

HEARING: March 21, 2011, at Toronto
Oral

DATE OF DECISION: March 25, 2011

NEUTRAL CITATION: 2011 ONWSIAT 722

DECISION(S) UNDER APPEAL: S. van Veen, Appeals Resolution Officer, June 29, 2010

APPEARANCES:

For the worker: G. Majesky, International Brotherhood of Electrical Workers (IBEW) Local 353

For the employer: Did not participate

Interpreter: N/A

REASONS

(i) Introduction to the appeal proceedings

- [1] The worker appeals the Appeals Resolution Officer (ARO) decision June 29, 2010. That decision denied the worker ongoing entitlement (including arthroscopic surgery on August 28, 1996) for a right knee injury occurring on October 19, 1995.

(ii) Issues

- [2] At the outset of the hearing, I discussed the issue agenda with Mr. Majesky. It was agreed that in this appeal, the Tribunal will decide: whether the worker has ongoing entitlement, including surgery in August 1996, under the claim established for the compensable right knee injury of October 19, 1995.

(iii) Background and testimony of the worker

(a) History to October 19, 1995

- [3] The worker testified that he was born in Germany and immigrated to Canada in 1966. In 1989, at age 27, he began an electrician apprenticeship and has continued to work in that trade to the present time. The worker testified that prior to 1995, his health was good and he enjoyed playing soccer and rollerblading. He stated that he did not have any other significant injuries and was not involved in any Motor Vehicle Accidents (MVAs).
- [4] On February 4, 1992, the worker injured his left shoulder at work. Information in the Case Record indicates that a claim file was established for that injury. At the hearing, the worker recalled this injury. He recalled that he may have lost one or two days from work as a result of the injury.
- [5] On July 9, 1995, the worker was hired through his union hiring hall by the accident employer, a construction company. He was employed as an electrician. The worker testified that he was in good health when he was hired by the accident employer. He recalled that he was responsible for installing pipes, devices and cylinders at a plant that produced lipstick and nail polish. His work often required him to climb extension and step ladders.
- [6] On September 24, 1995, he suffered a non-compensable injury to his right knee when he was kicked during a soccer game. He attended the Emergency Department of a local hospital the next day. In a report dated September 25, 1995, the attending physician at the hospital advised that the worker presented with tenderness on the distal side of his right knee and painful range of motion. An x-ray scan was reported to be negative. The worker was discharged into the care of his family doctor with a diagnosis of acute knee sprain and he was given a tensor band.
- [7] The worker initially testified that after being kicked by an opposing player, he continued to play for a little while, but then stopped playing before the end of the game. However, later in his testimony, he stated that he completed the game, but opted not to play a second game on the same day. The worker characterized the pain resulting from the kick as "2 out of 10" on a pain scale with 10 being extreme pain. The worker recalled that he returned to work the next day and did not lose anytime from work as a result of the non-compensable accident. He stated that he

returned to his regular duties which included climbing ladders. He stated that there was no instability or giving way of his right knee.

- [8] Mr. Majesky showed the worker pay information in the Case Record produced by the accident employer which indicates that he lost about one week from work as a result of the non-compensable injury to his right knee. The worker stated he did not recall losing time from work. He added that he was somewhat nervous in the Tribunal hearing. He did recall being given a tensor bandage and wearing it for a short duration after the soccer accident. The worker testified that he did not follow-up with his family doctor as recommended by the Emergency Room physician because the symptoms in his right knee improved.

(b) History from October 19, 1995

- [9] On October 19, 1995, the worker fell from the third rung of a ladder at work and injured his right knee. At that time, he was 33 years old.

- [10] There are no contemporaneous medical reports respecting this accident. However, in a report dated November 11, 1996, Dr. T. Campbell, family doctor, advised that the worker was initially seen by Dr. Chong, another family doctor in the same clinic, on October 20, 1995. Dr. Campbell advised:

[The worker] described slipping off a ladder the previous day at work. He complained of knee stiffness, pain and swelling. Dr. Chong found a medial effusion, decreased flexion to 130 and joint line tenderness. No ligamentous nor cruciate instability was found, but with early joint injuries this is difficult to assess due to pain. A diagnosis of knee sprain was made and ice, elevation and two weeks of Naprosyn # 375 mg 2x daily was prescribed. There was no mention of work restrictions documented.

- [11] The worker offered details with respect to the mechanism of injury. He testified that at the time of the compensable accident, he was installing one-inch conduit piping. To do this, he stood on the third rung of an extension ladder which was supported at the top end by a wall. The worker testified that the accident occurred when his right foot slipped off the third rung causing his right leg to go through the third and fourth rungs. He testified that his body twisted and he fell backwards striking his shoulders on the ground. He stated that his "right leg got hung up on the third rung". The worker testified that he heard and felt a "pop" in his right knee followed by immediate and extreme pain. The worker characterized the pain in his right knee as 8 or 9 out of 10. The worker testified that he was helped to his feet by a co-worker and that he reported the injury immediately to the employer.

- [12] In an Employer's Report of Injury/Disease (Form 7) dated October 25, 1995, the employer advised that the worker reported slipping off the third step of a ladder and falling onto his right knee. The employer emphasized that the worker had injured his right knee in a soccer game only a few weeks earlier.

- [13] According to an Employer's Continuity Report dated October 7, 1996, the worker did not lose time from work as a result of the compensable accident and continued in his regular duties until a general layoff on December 14, 1995. The worker did not file a claim with the Board until October 1996.

- [14] I questioned the worker about his lack of immediate reporting to the Board given the fact that he had prior experience communicating with the Board for his 1992 left shoulder injury. The worker could not recall why he did not contact the Board immediately and seemed surprised at the hearing that his first contact with the Board after this compensable accident was almost one year later.
- [15] The worker testified that he lost three or four shifts due to the pain in his right knee and that the employer nevertheless paid him his regular wages even though he did not work. He recalled that when he returned to work, he was offered a brief period of light duties consisting of working with a clay mix for devices. He stated that he was not required to climb ladders while performing modified duties. The worker testified that he eventually returned to “termination duties”, which he had performed on occasion prior to the compensable accident.
- [16] Mr. Majesky questioned the worker about a gap in seeking medical attention between October 20, 1995 and February 1996. The worker testified that during this period, he treated his right knee with ice and consumed anti-inflammatory medication. He stated that he generally avoided the medical community. The worker testified that although he did not seek medical attention between October 1995 and February 1996, his right knee continued to be symptomatic. He stated that it gave way on him on several occasions. He stated that whenever he twisted or pivoted on his right knee, he would “go down”. On other occasions his knee would “lock up” while walking.
- [17] In his report of November 11, 1996, Dr. T. Campbell advised that following the worker’s visit with Dr. Chong on October 20, 1995, the worker was not seen again until February 19, 1996. At that time:
- ...he presented with a right knee injury after slipping on ice 3 days earlier. Dr. Chong noted an effusion and joint bone tenderness and he questioned internal knee damage. After continuing pain, a referral to a sports medicine clinic...was made in early May. A diagnosis of an ACL tear was given May 9, 1996.
- [18] The worker emphatically denied slipping on ice in February 1996 or at any other time. He testified that his right knee gave way in February 1996 as it had on previous occasions since October 19, 1995. However, he explained, in February 1996, the pain was sufficiently significant to seek medical attention. He characterized the level of pain as “9 out of 10”.
- [19] At the hearing, the worker recalled that he returned to work in the spring of 1996. Information in the Case Record indicates that he returned to regular duties on or about April 19, 1996 with a different employer.
- [20] On July 8, 1996, the worker was seen by Dr. Campbell again. In a report dated August 6, 1996, he recorded the worker’s history of injuring his knee one year earlier after falling off a ladder and now suffering from “recurrent knee problem” in the form of joint instability with twisting. He advised that the worker complained of recurrent exacerbation of his right knee and instability. The family doctor confirmed the diagnosis of anterior cruciate ligament (ACL) tear and noted that surgical intervention was being considered. The worker testified that in early July 1996, while cleaning his apartment, he “stepped the wrong way” and

his right knee gave out on him. It was for that reason he saw Dr. Campbell and also stopped working.

[21] In a report dated August 6, 1996, Dr. W. Kraemer, orthopaedic surgeon, advised that the worker suffered an injury to his right knee while playing soccer, but the exact nature of the injury was not clear. He advised that the worker had swelling, but was able to resume full activities shortly thereafter. Dr. Kraemer noted the worker's injury resulting from a fall from a ladder. He understood that this was a "more significant injury to [the worker's] right knee" and that his knee "popped" followed by significant swelling and pain. Dr. Kraemer understood that the worker was able to return to work and performed light duties for four days before resuming regular duties. Dr. Kraemer advised that since the injury at work, the worker had three or four episodes of his knee giving out on him followed by periods of swelling. He noted that the worker stopped working on July 1, 1996 because of another episode of his right knee giving out and pain. On examination, Dr. Kraemer opined that the worker suffered an ACL tear and possibly a meniscal injury.

[22] On August 28, 1996, the worker underwent an arthroscopy of his right knee, right lateral menisectomy (partial) and arthroscopic anterior cruciate ligament reconstruction with bone patellar and bone autograft. In the surgical report, Dr. Kraemer recorded the following history:

This 33 year-old man had an injury to his right knee playing soccer a year ago. The symptoms in the knee resolved, however, he reinjured his knee when he fell off a ladder at work several months ago. Following this he had numerous episodes of knee locking and giving way, symptoms suggestive of pivot shift. He was rehabilitated with physiotherapy, however, he was still symptomatic in his right knee.

[23] In a post-operative report dated September 9, 1996, Dr. Kraemer advised that the worker was doing well and had started physiotherapy.

[24] On October 3, 1996, the worker completed Worker's Report of Injury/Disease (Form 6). He advised that he fell off a ladder and injured his right knee. He advised his right knee was "sore for 3 days" following which he thought it was better and he was placed on light duty.

[25] Also on October 3, 1996, the worker completed a Worker's Continuity Report. In this document, he advised that he started a new job on April 19, 1996, and experienced persistent swelling and soreness in his right knee.

[26] In a report dated October 7, 1996, Dr. Kraemer advised that the worker would be ready to return to light duties beginning in November 1996 and regular duties in December 1996.

[27] On October 28, 1996, a Board Claims Adjudicator (CA) spoke to the worker on the telephone and recorded their conversation in Memorandum #3:

The worker states that in or around early August 1995, he was playing soccer when another player hit him from side. He recalls that he continued to play and did not feel any acute discomfort. The next day, his right knee was swollen and uncomfortable. He sought medical attention from his then family doctor, Dr. Chong. He was told it was minor and to take some anti-inflammatories. The worker continued to work and did not have any further problems until 19 Oct 1995.

On 19 October 1996 [sic] the worker states that he fell from the third step of a ladder landing on his right knee. He states that the onset of pain was immediate. He sought medical attention the next day and recalls that he was off work for two days or so. He states that he was paid by the employer.

Although the...employer states that the worker had performed regular duties from October 1995 – December 1995, the worker states that this was not true. He states that he was working in “termination” and that his then foreman [D] could confirm this.

After he was laid off due to lack of work and had the winter to rest, he was fine when he resumed pre-accident duties in April 1996. He recalls that although he experienced occasional instability, he was able to work until 02 Jul 1996 when the knee “popped out” without incident.

[28] On October 29, 1996, the CA spoke to ER, the employer’s representative, and documented their telephone conversation in Memorandum #4. ER advised that the termination work performed by the worker was “not provided...as a special modified job” and that the worker performed termination duties prior to the compensable accident.

[29] In a report dated November 3, 1996, Dr. Kraemer advised that although the worker was progressing well, he experienced some “locking sensation” with prolonged standing. On examination, the worker was found to have a mildly positive Drawer test and Lachman test indicating that the surgical repair had “stretched a little bit”. Dr. Kraemer advised that this finding was not uncommon and that the worker should probably resume full duties by the end of November.

[30] On November 28, 1996, the CA contacted the worker again and documented their telephone conversation in Memorandum #7. At that time, the worker advised that he did not lose time from work as a result of the compensable accident of October 19, 1995, but consumed anti-inflammatory medication. The worker stated that on February 16, 1996, while off work, his right knee gave way and fell to the ground. He suffered acute pain and sought medical attention from Dr. Chong. He also recalled an incident occurring at work on May 27, 1996, in which he stepped “oddly” on something on the ground and experienced pain for which he sought medical attention the following day.

[31] In a report dated December 11, 1996, Dr. T. Rindlishbacher, medical doctor and physiotherapist, advised that the worker was treated between May and August 1996. During that time, the worker demonstrated varying degrees of swelling and reduced range of motion in his right knee. He advised:

Diagnosis on May 24/96

- Grade II – III tear of anterior cruciate ligament of the right knee
- possible meniscal tear of right knee

On all other dates

- Grade III tear of the anterior cruciate ligament of the right knee
- possible meniscal tear

[32] Dr. Rindlishbacher advised that on May 24, 1996, the worker reported four episodes of his knee giving way leading to swelling lasting about two days. He advised that one episode occurred at work in the winter of 1996 when the worker “slipped on ice”. Dr. Rindlishbacher

advised that on July 5, 1996, the worker presented with a history of walking with friends four days earlier when his right knee “severely gave way” causing him to fall to the ground.

- [33] In Memorandum #20, dated April 7, 1997, Dr. Bishop, Board Unit Medical Adviser, reviewed the evidence in the worker’s claim file and opined:

Although an anterior cruciate ligament injury could occur with either mechanism of injury, a soccer injury would appear to carry a greater likelihood of this [type] of damage, than a slip off a ladder. There was one week’s lost time with the soccer injury, and only a brief [period of] modified duties with the compensable injury. Noting also only the single medical visit after the compensable accident, in my opinion the information on file does not indicate a causal relationship between the Oct. 19/95 accident and the anterior cruciate disruption.

- [34] On July 10, 1998, Dr. Kraemer advised that the worker was doing quite well and remained reasonably active in sports and roller-blading. However, the worker reported that on occasion he wore a brace for sports. An x-ray taken at that time did not reveal any abnormalities.

- [35] In a report dated April 27, 2009, Dr. Kraemer advised that the worker and his union representative (Mr. Majesky) attended his office. Dr. Kraemer advised that he reviewed his notes which indicated:

[The worker] had an injury to his right knee in October 1995. He fell off a ladder. His foot got caught in the third rung from the bottom and he twisted as he fell. He heard a pop. Following that, he had 3 or 4 more episodes of giving way and instability of his right knee, which eventually culminated in right knee ACL reconstruction surgery on August 29, 1996 [sic]. It is noted that he had had an injury to his right knee playing soccer sometime prior to his work injury, but he had recovered fully and had returned to full-time work and had had no episodes of giving way of his knee.

Based on the review of my notes and discussion with the patient, clearly the work injury mechanism is consistent with his ACL tear and certainly, clinically, all of his instability symptoms started after that fall. In my documents, it is clear that his work-related injury of October 1995 resulted in a tear of his ACL and his requirement for ACL reconstruction surgery in August 1996.

- [36] On February 18, 2010, the worker underwent arthroscopic surgery again for a right knee meniscal tear, grade III chondromalacia in the right medial femoral condyle and grade II chondromalacia patella.

(iv) Submissions

- [37] On behalf of the worker, Mr. Majesky reviewed the history of this case and medical reporting from treating physicians. He acknowledged the discrepancies in the claim file and delays on the worker’s part in pursuing entitlement with the Board. He suggested that the worker was not a good advocate for himself and only sought medical treatment when his symptoms became severe.

- [38] Mr. Majesky submitted that at no time did the Board acquire a full appreciation or understanding of the mechanism of injury on October 19, 1995. He emphasized the worker’s testimony that he twisted his knee when his right foot slipped on the third rung of the ladder at work. He emphasized the worker’s testimony of recurrent right knee problems thereafter and

Dr. Kraemar's express endorsement that the compensable accident was compatible with the worker's subsequent knee problems and resultant surgery for ACL reconstruction.

(v) Analysis and conclusions

[39] I have carefully considered all of the available evidence in the Case Record, testimony of the worker and submissions of Mr. Majesky.

1. Does the worker have ongoing entitlement, including the surgery on August 28, 1996, under the claim established for the compensable right knee injury of October 19, 1995?

[40] The accident at issue in this appeal occurred on October 19, 1995. Accordingly, the *Workers' Compensation Act* (WCA) applies as amended by the *Workplace Safety and Insurance Act* (WSIA).

[41] Pursuant to section 126 of the WSIA, the Board has identified *Operational Policy Manual* (OPM) Document No. 15-03-01, "In the Course of and Arising out of – Recurrences", which states:

Policy

A worker is entitled to benefits for a recurrence of a work-related injury or disease.

A recurrence may result from an insignificant new accident, or may arise when there is no new accident. To identify a recurrence, the WSIB must confirm that there is clinical compatibility between the original injury or disease and the current condition, or a combination of clinical compatibility and continuity.

...

Guidelines

Recognizing a recurrence

Clinical compatibility

To establish clinical compatibility, a decision-maker compares the worker's current clinical condition to that following the initial accident. The decision-maker considers

- whether the parts of the body affected now are the same as, or related to, those affected initially
- whether the body functions affected now are the same as those affected initially, and
- the degree to which body functions are affected now (as compared to the effect of the initial condition).

Similar clinical conditions indicate that the current problem or problems may be a result of the original injury, whereas dissimilar or unrelated clinical conditions indicate that there is no compatibility, and therefore no recurrence.

Continuity

To establish continuity (i.e., a connection between the original clinical condition and the most recent problem or problems), the decision-maker considers whether the worker has

- complained to supervisors, co-workers, or health care practitioners on an ongoing basis since the original injury
- demonstrated ongoing symptoms since the original injury

- required work restrictions or job modifications
- had ongoing treatment for the original condition
- or experienced a lifestyle change since the original accident (e.g., has the worker become unable to participate in household duties, or social or recreational activities?).

In complex cases, the decision-maker may consult with WSIB clinical staff to assist in making this determination.

[42] In determining whether there is a causal relationship between an injury and subsequent medical condition, Tribunal decisions have widely applied the significant contributing factor and balance of probabilities tests.

[43] In *Decision No. 280*, 6 W.C.A.T.R. 27, the Panel elaborated on the nature of “significant contributing factor”. They wrote at page 36:

A “significant contributing factor” is a factor of considerable effect or importance or one which added to the worker’s pre-existing condition in a material way to establish a causal connection.

[44] In *Decision No. 661/93*, 33 W.C.A.T.R. 64, the Panel considered the question of how to approach the issue of causation in a context where there may be a number of potential factors or causes including the compensable accident to explain the emergence of a subsequent medical condition. The Panel concluded at page 77:

...the Panel is required to apply [the evidentiary standard of] a balance of probabilities...[where] there are...several possible scenarios, [the] Panel must decide what the most probable scenario is.

[45] Two important facts are not disputed in this case. First, the worker suffered an injury to his knee on October 19, 1995 when he lost his footing on a ladder at work. Second, the worker underwent arthroscopic surgery to his right knee involving a right lateral menisectomy (partial) and anterior cruciate ligament reconstruction with bone patellar and bone autograft on August 28, 1996.

[46] Other important facts are disputed in this case. The worker disputes that he lost time from work as a result of the non-compensable soccer accident on September 24, 1995—despite being shown compelling evidence (payroll information) to the contrary. However, for the reasons set out below, this dispute is not significant in my view.

[47] The worker contends that he lost about four shifts following the compensable accident before returning to modified duties then ultimately to regular duties. The employer maintained that the worker did not lose any time from work as a result of the compensable accident. Indeed, the worker appeared to concede this fact in his discussion with the CA on November 28, 1996 (Memorandum #7). The CA noted the worker’s information that he “did not lose any time from work” after the compensable accident. However, payroll information indicates that between October 14 to 20, 1995, the worker worked 28 hours rather than his regular 37.5 hours. Thus, at most, it appears that the worker may have lost 9.5 hours of work during week of the compensable accident, but there is no objective confirmation of the reason for the lost hours.

- [48] The worker testified that he was provided with modified duties (termination duties) after missing a few shifts following the compensable accident. The employer advised that the worker was not provided with modified duties.
- [49] The worker testified that at no time did he ever suffer a slip and fall on ice. There is significant evidence to the contrary. In the narrative report dated November 11, 1996, Dr. Campbell advised that Dr. Chong's notes indicated that the worker presented on February 19, 1996 with a right knee injury "after slipping on ice 3 days earlier". In his report of December 11, 1996, Dr. Rindlishbacher advised that the worker presented on May 24, 1996 with a history of four episodes of his knee giving way and one episode in the winter of 1996 in which the worker "slipped on ice". Dr. Rindlishbacher mistakenly understood that the injury occurred at work. However, it is not contested that the worker was not working in February 1996. The dispute about whether the worker's knee gave way or whether he slipped on ice in February 1996 is discussed in more detail below.
- [50] The worker testified that ever since the compensable accident of October 19, 1995, he suffered from recurrent episodes of right knee instability. However, the first objective medical evidence that suggests a problem with the worker's knee giving way occurred on May 24, 1996 when the worker offered this history to Dr. Rindlishbacher. It is significant, in my view, that this history was provided after the February 1996 incident.
- [51] In determining whether a worker has suffered a recurrence, Board policy states that similar clinical conditions indicate that the current problem or problems may be a result of the original injury. Put another way, clinical compatibility between the original injury and subsequent condition may indicate a relationship between the two. However, in this case Dr. Bishop confirms that clinical compatibility exists not just between the compensable accident and subsequent diagnosis of ACL tear, but also between the non-compensable soccer accident and subsequent condition. I note as well that the diagnosis of ACL tear and suspicion of a possible meniscus injury occurred after the February 1996 incident. From a lay perspective, this incident is also be compatible with the subsequent diagnosis. In my view, the analysis of causation in this case cannot reasonably be limited to an assessment of clinical compatibility between the compensable accident and subsequent condition given the evidence that other non-compensable events may also be compatible and given the lack of objective contemporaneous confirmation of subsequent symptoms following the compensable accident for several months.
- [52] On the question of continuity, there is simply no objective contemporaneous medical or non-medical documentary evidence to support ongoing symptoms between the compensable accident on October 19, 1995 and the incident in February 1996. The worker's assertion that he lost four shifts from work and was provided with modified duties is not supported by the employer's evidence. I note that there is no co-worker or witness statement attesting to the worker's lost time from work, participation in modified duties or complaints of right knee problems. There is no evidence at all of any giving way episodes at work despite the evidence that the worker continued working in regular duties for another two months until being laid off due to a shortage of work.

[53] Considering all of the available evidence, I find that, on a balance of probabilities, the compensable accident of October 19, 1995 did not significantly contribute to the worker's subsequent right knee condition.

[54] In reaching this conclusion, I acknowledge the worker's testimony that he suffered from recurrent episodes of his right knee giving way ever since the compensable accident. This is the history he gave to Dr. Kraemer. I have already discussed the lack of supporting evidence on this point. There is simply no medical or non-medical contemporaneous support for his assertion. I agree with Mr. Majesky that the worker's overall presentation may have contributed to some confusion in this case about the exact history of injury and subsequent events. I accept that the worker did not intentionally set out to mislead either the Board or the Tribunal about the history of his right knee injuries. However, the worker's recollection of events stands in stark contrast with the available documentary evidence on important points. Namely, the lack of objective confirmation that after the compensable accident he lost time from work, participated in modified duties or experienced recurrent episodes of his right knee giving way. The first documentary reference to his right knee giving way occurs in the May 1996 notes of Dr. Rindlishbacher—seven months after the compensable accident.

[55] I acknowledge Dr. Kraemer's opinion that based on his review of his notes and discussion with the worker, the mechanism of injury in the fall from the ladder was clearly consistent with the worker's ACL tear and that all of his instability symptoms started after that fall. He further opined that the work-related injury of October 1995 resulted in a tear of his ACL and his requirement for ACL reconstruction surgery in August 1996. Although Dr. Kraemer may well have had a better appreciation for the mechanism of injury occurring in the compensable accident, he did not indicate awareness that the worker's history is not supported by the available contemporaneous documentary evidence.

[56] I am persuaded that the worker does not have ongoing entitlement for his compensable right knee injury specifically for the following reasons:

- Dr. Chong, October 20, 1995 (as summarized in Dr. Campbell's report of November 11, 1996) – His notes indicate that the worker slipped off a ladder on October 19, 1995 and presented the next day with complaints of knee stiffness, pain and swelling. Dr. Chong found a medial effusion, decreased flexion to 130 degrees and joint line tenderness. However, no ligamentous or cruciate instability was found. A diagnosis of knee sprain was made and the worker was advised to apply ice, elevate his leg and take anti-inflammatory medication for two weeks. At that time, there was no indication that any further diagnostic investigations were undertaken (i.e. x-ray) or any arrangement was made for a follow-up appointment or any referral was made to a specialist. There is no indication of any medical restrictions preventing the worker from performing the full range of his duties. Although Dr. Campbell advised that a more significant pathology might have been missed on October 20, 1995 due to the expected pain associated with a joint injury and inability to thoroughly examine the joint, identifying a more serious pathology was not a problem in February 1996 by the same treating physician (Dr. Chong) despite the worker's pain at that time which was also described as "9 out of 10".

- October 1995 to February 1996 – During this period, there is no objective contemporaneous evidence that the worker suffered from any problem with his right knee. There is no medical treatment and no supporting evidence of any work accommodation. There is no objective evidence of any episodes of the worker’s right knee giving way. This period resonates well with the available evidence summarized immediately above.
- The February 16, 1996 incident – The worker has steadfastly denied slipping on ice in February 1996. He reported (in May 1996) that his right leg gave out on that day. However, as noted above, the contemporaneous medical evidence from Dr. Chong indicates that on February 19, 1996, the worker “presented with a right knee injury after slipping on ice 3 days earlier”. Of significance, in my view, are the other observations recorded by Dr. Chong and recited by Dr. Campbell in the November 11, 1996 report:

Dr. Chong noted an effusion and joint bone tenderness and he questioned internal knee damage. After continuing pain, a referral to a sports medicine clinic...was made in early May. A diagnosis of an ACL tear was given May 9, 1996. (emphasis added)
- The clinical notes of Dr. Chong - Also of significance, in my view, is that Dr. Chong was well aware of the compensable accident in October 1995 having treated him at that time and in February 1996 made no mention of recurrent episodes of the worker’s knee giving way at all, much less as the cause of the injury on February 16, 1996. The documentary evidence after the incident on February 16, 1996, indicates that the worker was found to have internal knee damage. Persistent symptoms thereafter are confirmed in Dr. Chong’s notes which led the worker to further consultations with other specialists and a diagnosis of an ACL tear. This history strongly suggests a much more serious incident occurred on February 16, 1996 than either the soccer or ladder accidents and was not addressed by Dr. Kraemer.

[57] For the above-stated reasons, I find that, on a balance of probabilities, the compensable accident of October 19, 1995 did not significantly contribute to the worker’s subsequent knee disability and surgery in August 1996.

DISPOSITION

[58] The worker's appeal is denied.

DATED: March 25, 2011

SIGNED: S. Ryan